

SEVERITY

HIGH

CONFIDENCE

—

EMERGENCY OVERRIDE

CRITICAL — Life-threatening findings

TRIGGERED REASONS

- SpO2 critically low (88%) — respiratory failure risk
- Heart rate elevated to 112 bpm — possible sepsis
- Fever 103°F with rigors in elderly diabetic

IMMEDIATE FIRST AID

- Position patient upright to ease breathing
- Administer oxygen via face mask at 6-8 L/min if available
- Give paracetamol 1g orally for fever
- Do NOT give food or drink — preparation for possible procedures

Immediate ambulance transport to district hospital with oxygen support.

REFERRAL LEVEL HIGH

Facility District Hospital with ICU

Urgency Immediate (within 1 hour)

Transport Ambulance with oxygen and IV setup. If ambulance unavailable, private vehicle with portable oxygen cylinder and accompanying health worker.

RECOMMENDATION

Admit to ICU. Start empirical antibiotics, oxygen therapy, IV fluids, and investigations (chest X-ray, CBC, BMP, blood culture, sputum culture).

TRANSFER CHECKLIST

- Patient identity card and Aadhaar
- Previous medical records and current medication list
- Latest vitals chart with trend
- Voice/clinical notes from CHW
- Emergency contact numbers of family
- Allergy information card
- Insulin and diabetic medication supply for 24 hours
- Cash for hospital admission deposit

PATIENT INFORMATION

Patient Name	—
Age	—
Gender	—
Phone	—
Address	—

HIGH

Severity: HIGH — Urgent

Confidence: HIGH

Elderly diabetic patient with 3-day fever, productive cough, pleuritic chest pain, and SpO2 of 88%. Suspicion of community-acquired pneumonia with possible sepsis. Immediate referral to district hospital with oxygen support and empirical antibiotics is indicated. Vitals show tachycardia, low-grade

PARAMETER	VALUE	UNIT	STATUS
Blood Pressure	—	mmHg	—
Heart Rate	—	bpm	—
Temperature	—	°C	—
Oxygen Saturation (SpO2)	—	%	—
Blood Glucose	—	mg/dL	—

ANOMALY FINDINGS

- Critical: SpO2 below 90% (88%)
- Tachycardia: 112 bpm
- Fever above 102°F (103°F)
- Diabetic with elevated glucose (186 mg/dL)
- Pleural-type chest pain with productive cough
- Reduced urine output in last 24 hours

LABORATORY FINDINGS

No laboratory values recorded.

LAB ALERTS

1. Elevated WBC (14,800) — infection
2. Elevated CRP (186 mg/L) — severe inflammation
3. Elevated procalcitonin (1.8 ng/mL) — bacterial infection likely
4. Reduced eGFR (42) — CKD worsening
5. Elevated D-dimer (1.2) — needs PE workup

CLINICAL SUMMARY

Elderly diabetic patient with 3-day fever, productive cough, pleuritic chest pain, and SpO2 of 88%. Suspicion of community-acquired pneumonia with possible sepsis. Immediate referral to district hospital with oxygen support and empirical antibiotics is indicated. Vitals show tachycardia, low-grade fever, and borderline hypotension.

POSSIBLE CONDITIONS

1. Community-acquired pneumonia (severe)
2. Sepsis secondary to pneumonia
3. Acute respiratory failure (Type I)
4. Diabetic ketoacidosis (less likely)

RECOMMENDED ACTIONS

1. Administer high-flow oxygen to maintain SpO₂ > 94%
2. Start empirical IV antibiotics (Ceftriaxone 1g + Azithromycin 500mg)
3. Obtain chest X-ray and sputum culture
4. IV fluid resuscitation with normal saline
5. Strict input-output monitoring

REFERRAL RECOMMENDATION

Immediate ambulance transport to district hospital with oxygen support.

VOICE TRANSCRIPT / SYMPTOMS NOTES

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VOICE TRANSCRIPT / SYMPTOMS NOTES (CONTINUED)

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AI Disclaimer — This report is generated using AI-assisted triage support and should not replace professional clinical judgment.
Final diagnosis and treatment decisions remain the responsibility of licensed healthcare professionals.